

Chapter 5 – Attachment and Psychopathology (Part 2)

VI. Attachment and Addictions

Alcoholism and Substance Abuse

- The relationship between attachment status and substance use was investigated in a college student population, yielding scores on three attachment categories: close, (i.e., comfort with closeness to others), depend (i.e., feeling confident others will be there in times of need), and anxious (i.e., having a lot of worry about being abandoned or not loved).
 - Students in the categories close and depend smoked less tobacco. No association was found between attachment and alcohol and marijuana. However, when stress-related use was assessed, the use of all three substances was significantly correlated with anxious attachment.
 - Authors also found that insecure attachment was associated with lower self-esteem and greater endorsement of dysfunctional attitudes.
 - The authors concluded that “insecure attachment, specifically anxiety over possible abandonment (anxious attachment), proved to be significantly related to both frequency of substance use and stress-motivated use of substances” and commented that “a plausible etiological process linking insecure attachment and substance use is that insecurely attached individuals develop dysfunctional attitudes about themselves such that when these underlying insecurities are activated, they deplete the individual’s self-esteem. Such low self-esteem enhances the likelihood of more drug use and, perhaps more importantly, more stress-motivated use of substances.”
- In a study of adolescents who had been psychiatrically hospitalized at age 14 as compared to demographically matched high school students without psychiatric histories, they found that at age 25, insecure attachment was significantly associated with the total number of incidents of hard drug use in later adolescence, namely, the use of heroin, cocaine, hallucinogens, amphetamines, and/or tranquilizers.
 - Certain dismissing states of mind, namely, high derogation of attachment and lack of memory on the AAI, were significantly related to higher drug use, as was unresolved status with respect to trauma, although unresolved status with respect to loss was not.
- In a study described earlier in the section on affective disorders and suicidality, adults classified as unresolved with respect to trauma, but not with respect to loss, were significantly more likely to report a history of alcohol or drug abuse, while those who were unresolved in relation to loss were more often reported to be involved in criminal behavior.
- A study examined psychiatrically hospitalized adolescents with affective disorder and/or conduct disorder. All participants were administered the AAI, which scored using a four-way category system: 38% dismissing, 2% autonomous, 42% as preoccupied, and 18% as unresolved. Dismissing attachment was linked to conduct disorders, preoccupied attachment was linked to affective disorders.

- Adolescent substance abuse was most prominent in the dismissing classification group. Among the 24 participants classified as dismissing, 16 reported problems with alcohol or drug abuse; among the 28 identified as preoccupied, 10 reported problems with alcohol or drug abuse.

- Authors note that “Adolescents using a dismissing attachment organization rely on an attachment strategy that minimizes distressing thoughts and affects associated with rejection by the attachment figure . . . Conduct disorder or substance abuse, in which overt denial or downplaying of distress, coupled with actions that display those distressing affects, did occur in the context of a dismissing attachment organization.”

- Other researchers noted affect dysregulation problems in drug addicts. They recognized that drug addicts are prone to failures in affect regulation and modulation, and have deficits in mentalizing.

- A researcher outlined an approach to addictions based on attachment treatment, and pointed out that “Unmet developmental needs” drive all addictions, and all addictions are “misguided attempts at self-repair.” The aim of attachment-oriented treatment of addictions, therefore, is the development of healthy internal working models for attachment and the associated healthy forms of affect regulation that can help addicts to detach from their objects of addiction.

- Studies that address the relationship between attachment classification and substance abuse have significant clinical implications, highlighting the importance of considering early attachment relationships and current attachment patterns when treating patients with addictions and substance abuse disorders.

Eating Disorders

- Researchers gave the AAI to 30 subjects with clinically significant eating disorders (ED) and 30 matched nonclinical controls. Of the ED patients, 10 had anorexia, 10 had bulimia, and 10 had a binge ED. The majority of ED patients (90%) had insecure attachment, mostly of the dismissing type (47%). The frequency of cannot classify (CC) was low (16%).

- The authors state: “In ED subjects, the relationship with the mother appears to be the most problematic, since it conveys a loving and at the same time role-reversing, rejecting and neglecting stance associated with two problematic attachment states of mind; a stance toward idealization and a stance toward involving anger.

- The ED patients, as compared to the healthy control subjects, had significantly greater parental rejection, involving, and neglect on the AAI parenting scales. ED patients as compared to controls had a significantly higher lack of recall regarding attachment experiences and significantly lower coherence of transcript and coherence of mind.

- Unresolved status with respect to trauma was also overrepresented in this ED sample, but only in the bulimic and binge ED groups and not the anorexic group.

VII. Attachment and Personality Disorders

Personality Disorders in General

- A pilot study to investigate the relationship between attachment styles and personality disorders, using the Experience with Close Relationships scale (ECR), showed positive correlation between ECR Attachment Anxiety scale scores and avoidant, dependent, obsessive-compulsive, paranoid, schizotypal, histrionic, narcissistic, and borderline personality disorders. Particularly strong correlations were found for borderline and dependent personality disorders.
- In a study of attachment and PTSD, data was also obtained about the association of attachment status and personality functioning. The results showed significant correlations between attachment status and presence of a personality disorder. The author states: "basic personality structure, including more dysfunctional manifestations, was not associated with abuse characteristics but was instead predicted by adult attachment."

Borderline Personality Disorder

- Individuals meeting the criteria for borderline personality disorder (BPD) suffer from a range of devastating behavioral problems, such as self-injurious behavior (69% to 75% prevalence) and a risk of suicide that is 400 times greater than in the general population. Those who suffer from BPD are often difficult for clinicians to treat due to their rapidly shifting attachment strategies and extreme ambivalence in forming attachments.
- Researchers studied the patterns of attachment and psychiatric status with inpatients and participants in a control group using the AAI. They found that the most specific and robust relationship was between BPD and attachment classification.
 - The BPD patients' AAI interviews were differentiated by a combination of three characteristics: (1) They showed a higher prevalence of abuse and neglect (89%), (2) they had significantly lower ratings on the Reflective Self-Function (RSF) scale, and (3) they scored significantly higher on the Lack of Resolution of Abuse or Loss scales (89%).
 - The RSF scale was a good predictor of infant-parent attachment security: Both mothers and fathers who were rated high in this capacity were three to four times more likely to have secure children. Those diagnosed with BPD reported parents who were less loving and more neglecting. The authors posited that individuals who had severe maltreatment in childhood may have responded by inhibiting their mentalizing function in order to defend against the realization that their caregiver wished them harm. They suggested that the initially defensive disruption of the capacity to depict feelings and thoughts in self and others becomes a core symptom in BPD, which makes it more difficult for these patients to work toward resolution of their traumatic stress.
- In the orphanage study, insecure attachment (mostly disorganized) as compared to secure attachment was significantly related to a mixed or borderline personality disorder (PD) in adulthood. Past or current trauma was not significantly related to a mixed or borderline PD diagnosis. These findings suggest that mixed and borderline PD are more related to early attachment disorganization than to trauma or abuse in later childhood or adulthood.

Conduct Disorder and Antisocial Personality

- The AAI was given to subjects incarcerated involuntarily on forensic psychiatric units after committing serious crimes such as murder or sexual offenses. The distribution of attachment classifications for the 40 subjects using the five-way system were Ds = 9, F = 2, E = 8, and Ud/CC = 21. Secure attachment was “strongly under-represented” and disorganization (Ud/CC) was “strongly over-represented” in these forensic subjects.

- With respect to personality disorder, the “total number of personality disorder symptoms was significantly related to attachment insecurity” and “the CC subjects have almost all been raised in institutional care, and they appear to show most personality disorders compared to the other attachment groups.” They add that sadistic and self-defeating personality disorders were especially prevalent in subjects with a CC classification combined with unresolved (Ud) status.

VIII. Conclusions About Attachment Status and Psychopathology

- While it is very clear that insecure attachment in general, and unresolved trauma or loss in particular, is associated with a wide range of psychiatric psychopathology, it is also clear that even with a large AAI database, consistent relationships between specific types of insecure attachment and specific psychiatric disorders are harder to establish.

- One limitation across this area of research has been the failure to use a five-way or even four-way classification model in many of the studies (leaving out CC and/or Ud). Trying to force the best fit into a secure, dismissing, or preoccupied category obscures the fact that in forensic and psychiatric settings, cannot classify (CC) may be the best fit for the clinical case.

- The CC classification represents pervasive disorganization across the AAI transcript, not topic-specific disorganization as in the case of unresolved status (Ud). It represents the worst of early parent–child disorganized attachment and is strongly associated with mixed or borderline personality disorder and major dissociative disorders. Therefore, disorganized attachment over the course of development has major implications for mental health and needs to become a main focus of future research. The authors of this book suggest such research would benefit from refining the CC category into a range of subtypes that can further differentiate the phenomena of attachment disorganization.

- It is not unusual for “treatment-resistant” patients to have made multiple attempts at finding appropriate treatment and suffered through shifting diagnoses, different recommendations for treatment strategies, and reactive medication management. These are the patients who as children had to adaptively develop a mix of attachment strategies due to inadequate and/or harmful parenting behaviors, which often underlies the disturbance presenting for treatment. The authors of this book believe that use of the Main and Goldwyn five-way AAI classification system, or the DMM system from Crittenden, allows for more accurate and detailed identification of these important underlying patterns.

VIV. The Orphanage Study: Attachment and Complex Trauma

- The senior author of this book, Daniel Brown, conducted a large number of forensic evaluations of adult survivors of childhood sexual abuse over many years in the context of civil lawsuits against alleged offenders and/or third parties. At one point, Dr. Brown was asked to evaluate a number of adult

survivors of childhood physical abuse (PA) and child sexual abuse (CSA) that had allegedly occurred at a Catholic orphanage in the greater New Orleans area in the late 1950s and 1960s as part of a lawsuit against the local Catholic diocese.

- Each of the priests had been previously accused of child molestation at least once, and the “solution” by the Church was to transfer all of these accused priests to work in the same orphanage. Extensive physical and sexual abuse of many orphans occurred in that environment.
- Dr. Brown conducted a detailed forensic psychological assessment of each alleged victim. The assessment consisted of, on average, 16 hours of face-to-face interviewing and psychological testing.
- The sample is unique in that all 17 subjects were abused in the same way, by the same group of offenders, during the same time frame. About 60% of the 17 orphanage victims were determined to have insecure attachment, mostly of the disorganized type (Ud and/or CC status), from the AAI scoring. Many of these subjects came from chaotic family backgrounds, characterized by, for example, repetitive violence and alcoholism in the home and/or a parent running a drug ring and/or a prostitution network out of the home.
- The other 40% of the group came from large, loving Catholic families and were likely to have secure attachment status due to healthy parenting. However, a common occurrence in these families was that the father had a serious industrial accident resulting in serious physical injury and disability or death; as a result, the mother was unable to afford to continue raising the children, and the family dissolved and the children were sent to the Catholic orphanage.
- The unique features of this sample allowed the possibility of separating out the relative differential contributions of early attachment status on the one hand, and the effects of childhood physical and sexual abuse in later childhood on the other hand, to the subjects’ overall adult psychopathology.
- To increase the statistical power, they also examined a second mixed group of CSA victims. Group A comprised the 17 subjects described above, and Group B consisted of 28 adult subjects, all of whom were also adult survivors of sexual abuse. Dr. Brown had also conducted similar forensic examinations on each of these subjects as part of a lawsuit.
- The main focus of the research was to determine how AAI-based attachment classifications were related to various diagnoses. The majority of insecure subjects on the AAI had disorganized status (Ud or CC classification) and many of the secure subjects also were disorganized with respect to trauma or loss (Ud).
- A relationship between insecure attachment and PD/BPD, dissociative disorders and bipolar type 2 was confirmed. There were no significant statistical relationships between insecure attachment and major depressive disorder, dysthymia, panic disorder, or somatoform disorder. Substance and alcohol abuse, as well as PTSD, were not statistically significantly related to insecure status.
- Authors note it is very clear from the study that Axis I conditions such as PTSD and depression, anxiety-spectrum conditions such as panic, and alcohol or drug abuse or dependency are significantly related to acts of physical and sexual abuse per se. It is also very clear from the orphanage study that a mixed or borderline personality disorder diagnosis, multiple dissociative disorder diagnoses, and a bipolar type II diagnosis are significantly related to insecure attachment and low coherence of discourse on the AAI.

- These findings linking psychopathology with insecure attachment can be seen to show the added vulnerability brought by insecurity and unresolved status (Ud) regarding past trauma or loss or, put the other way around, the diminished importance of unresolved status (Ud) when the underlying or overall pattern of attachment is secure. (Insecure subjects were more vulnerable to certain disorders even though both secure and insecure had Ud status).

- The results of the orphanage study have important implications for the treatment of complex trauma. It is very common for patients diagnosed with complex trauma to have multiple, comorbid psychiatric diagnoses such as PTSD, panic, generalized anxiety disorder, major depression or dysthymia, a somatoform disorder, a dissociative disorder, a personality disorder, and/or multiple addictive behaviors.

- As the term “complex trauma,” suggests, many have assumed that traumatic acts play a main role in the development of complex trauma disorders. This trauma-focused bias obscures the extent to which early attachment disturbance may account for complex trauma and/or multiple comorbidities, independent of or in combination with abuse.

- Simple PTSD is associated with single or cumulative specific trauma events. In addition to the simple PTSD manifestations, complex trauma-related disorders can also include somatoform, PD, dissociative, and often multiple addictive behaviors. Complex trauma is primarily associated with *insecure attachment aggravated by abuse or trauma in later childhood or adulthood*. There is little support for the view that complex trauma is a function of early trauma per se or a function of multiple-event trauma or cumulative abuse.

- These findings have major implications for the treatment of complex trauma-related disorders. In the view of the authors of this book, a consistent focus on trauma processing per se in patients with complex trauma-related disorders runs the risk of aggravating disorganization of mind, which is a feature of insecure attachment.

- Dr. Brown had the opportunity to serve as an expert witness in approximately 70 malpractice lawsuits against clinicians for allegedly implanting false memories of abuse in patients. He found that most of the defendant clinicians were not using risky memory recovery techniques to zealously recover or suggest abuse memories in these patients. Instead, Dr. Brown observed in many of these cases, systematic trauma processing with patients who had disorganized attachment aggravated by abuse in later childhood often led to further disorganization of mind.

- The authors of this book found that by using attachment-based treatment, complex trauma patients achieved high coherence of mind (on the AAI) and sometimes resolution of unresolved (Ud) status (on the AAI) without having done any type of trauma processing. Furthermore, once these patients had achieved high coherence of mind using the attachment-based treatment, it was easy to process the trauma with simple cognitive-behavioral exposure-based methods in a way that did not lead to disorganization of mind. “It is our hope that trauma clinicians will use this book to reconsider their approach to the treatment of trauma patients when there is a clear history of disorganized attachment.”

- The next section of this book addresses the treatment of attachment disturbances. Chapter 6 presents a review of the available attachment-based treatments. Chapter 7 describes the fundamentals of the model and methods of the Three Pillars approach to treatment, and Chapters 8, 9, and 10 detail the principles and practices of each of the Three Pillars.